Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

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Submitted electronically via www.regulations.gov

RE: Comments to Health Technology Ecosystem Request for Information (CMS-0042-NC)

Dear Administrator:

Albertsons Companies ("**AC**") welcomes the opportunity to respond to the Center for Medicare & Medicaid Services' ("**CMS**") request for information (RFI) on the digital health products market for Medicare beneficiaries and the current state of data interoperability and health technology infrastructure. Our RFI response addresses the current state of data interoperability and health technology infrastructure and how key stakeholders – patients, technology vendors, data providers, and networks – intersect across the following tenets identified by CMS:

> "Driv[ing] the development and adoption of digital health management and care navigation applications; Strengthen[ing] interoperability and secure access to health data through open, standards-based technologies; Identify[ing] barriers preventing the seamless exchange of health information across systems; and Reduc[ing] administrative burden while accelerating progress toward value-based, patient-centered care."

Our response focuseson daily engagement, nutrition, and behavioral/lifestyle data in ensuring that "health information is not locked away in silos but rather flows securely to where it is needed most—empowering patients and providers." Using learnings and successes from AC's Sincerely Health platform (the "**Platform**") we recommend that CMS incorporates relevant data sources and creates standards for disease-specific lifestyle and nutrition recommendations by providers and enables a universal easy—to-use application for patients that combines medical records, actionable lifestyle and care navigation recommendations for patients. In addition to leveraging nutrition data from grocery stores and lifestyle data from various connected devices, we recommend that CMS provide incentives for community pharmacies to more broadly participate in point-of-care data exchange, to support the transition to Value-Based Care. The basis for this recommendation is as follows: 1) access (approximately 90% of the U.S. population lives within five miles of a community pharmacy and most pharmacies offer extended hours compared to a traditional site of medical care), 2) trust (pharmacists consistently rank high in surveys measuring public trust in professions, often alongside nurses and doctors), 3) frequency (on average, Medicare beneficiaries

visit their pharmacy at least once a month, to pick up prescriptions or to receive point of care services) 4) scale and speed (as evidenced by COVID response, community pharmacies are equipped to respond to national care demands including testing/screening, dispensing and prescribing (where allowed)). Our response also builds on the success of initiatives including Blue Button 2.0, CMS' Interoperability Final Rule, and the Make America Health Again ("**MAHA**") initiative, while also reducing the risk of waste, fraud, and abuse.

About Albertsons

AC is a national grocer operating integrated pharmacy locations across 34 states plus the District of Columbia. The company serves 36 million customers weekly through 2,200 grocery stores and millions of patients through its pharmacy operations. Among these, millions are Medicare beneficiaries who utilize both grocery and pharmacy services. Through its Sincerely Health platform, AC extends beyond traditional retail to connect customers with broader healthcare and wellness services, creating a seamless, comprehensive customer experience.

About Sincerely Health

AC's Pharmacy and Health division operates the Platform, a consumer-facing health and wellness mobile application designed to help our customers eat better, move more, and live healthier through science-backed insights. The Platform combines personalized tools and guidance for nutrition, physical activity, and medication management, along with a broad range of pharmacy services. The Platform also connects users to medical nutrition therapy, medically tailored grocery fulfillment, and primary and urgent care providers. It leverages a wide range of wearable activity and medical device data streams for trend monitoring and insights. The Platform integrates data from a customer's grocery purchases, connected wearables, surveys, and pharmacy records ("**Additive Sources**") to enable smarter food choices and improve their overall health. The Platform has reached millions of enrollments since its launch (2023), while demonstrating high levels of engagement thanks to its gamification and rewards system. The Platform consolidates data from the Additive Sources (age, sex, weight, history of chronic diseases, nutrition, and lifestyle habits) to display a "health score", an actuarially derived mortality score. The Platform encourages small daily changes to improve wellness habits, rewarding users with "healthy" points that contribute to grocery discounts and non-financial incentives, such as badges.

Based on aggregated data gathered from Platform's grocery/retail pharmacy users, the following results were documented:

- Demonstrated improvement in physical activity levels
- Increased compliance with Dietary Guidelines for Americans (MyPlate)
- 1.2x higher engagement among Medicare, 1.7x among Medicaid users vs. shopper mix
- Improvement in health score

Within the context of the RFI, the Platform is a digital health product, with functionalities that fall within health management, care navigation, and personal health record definitions outlined by CMS.

Responses to Selected Patient Topics

Patient Needs

PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?

Response: Poor nutrition drives chronic disease and inflates healthcare costs as documented in U.S. Center for Disease Control and Prevention studies linking chronic disease and increased health care spending to poor nutrition (See: https://www.cdc.gov/nutrition/php/healthy-food-environments/index.html).

The challenge lies in bridging two separate worlds: clinical nutrition guidance provided in health care settings (e.g., primary care encounters) and consumer food purchasing decisions made in grocery stores. As our healthcare systems increasingly prioritize population health and preventable disease reduction, coordinating becomes essential to meaningfully impact health outcomes. Digital health products that provide actionable insights related to optimal nutrition, physical activity, and other wellness recommendations with bi-directional data flows between patient, provider, and permissions for caregivers should become a staple in chronic disease care for Medicare populations.

PC-4. What features are missing from apps you use or that you are aware of today?

a. What apps should exist but do not yet? Why do you believe they do not exist yet?

Response: A unified digital health product, that connects health care data (e.g., Medicare claims data across providers and Blue Button 2.0) with real-time consumer-focused nutrition guidance, insights, and incentives from retailers) at the point of purchase at grocery stores would translate clinical recommendations into actionable shopping decisions. These recommendations and decisions can enhance the existing claims and demographics data shared through Blue Button 2.0.

As CMS recognized in the solicitation summary, current solutions, such as digital health products, are fragmented and siloed. There are digital health products that exist for certain companies with credentials and data for a specific provider or vendor. In fact, the Platform is an example of a

solution specific to only AC shoppers. No digital health product bridges the gap between what medical professionals recommend and what patients buy or incorporate into their daily routine. This approach, when expanded to grocery stores and integrated with Medicare claims data, would coordinate clinical recommendations with patient food purchasing behavior. We believe an extension of this concept across grocery stores when paired with Medicare claims data, is a novel concept that CMS is uniquely positioned to address. Specifically, analyzing expenditure data on Rewards and Incentives programs and Supplemental Benefits offered by Medicare Advantage, in tandem with claims data and patterns in behavioral choices that beneficiaries make, would enable real-world measurement of how Medicare non-medical benefits can impact chronic care outcomes at scale.

b. What set of workflows do you believe CMS is uniquely positioned to offer?

Response: CMS is uniquely positioned to establish comprehensive care coordination workflows that manage the coordination of data for Medicare patients across the continuum of the patient journey, as evidenced by the success of the Blue Button 2.0 program. For example, the Medical Nutrition Therapy (MNT) Act will effectively provide MNT coverage for all Medicare patients. CMS will have both clinical nutrition data and the regulatory authority to facilitate consumer-based data integration. CMS can design and deploy targeted financial incentives and other approaches that encourage patients to purchase meals and groceries aligned with clinical recommendations. This includes benefit adjustments, supplemental nutrition assistance, or similarly, with patient consent the data may be used for predictive analytics and population health management.

Additionally, community pharmacies can provide expanded access to preventive screenings and services, providing early risk signals (such as A1C, blood pressure or cholesterol measurements or other point-of-sale discounts for care data) that can and should be leveraged as CMS moves to Value-Based Care.

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

Response: Related to the response in PC-4, CMS has a vested interest in the success of digital health products. Successful digital health products will lead to better population health and reduced health care spending. Some of the relevant criteria would include: (a) ability to use the Platform

across a variety of consumer-based settings , (b) integration of health care provider and other sources of information to provide education and guidance, (c) real-time feedback and scoring to help the patient make adjustments, (d) a plan to promote adoption and continued interaction with the Platform, including monetary incentives, (e) proven track record of adoption, consumer-grade engagement and behavior change and outcomes and (f) measurable data and insights to continue to improve the Platform and key objectives for the patient population.

b. What changes would enable timely access to high quality CMS and provider generated data on patients?

Response: A CMS sponsored and funded digital health product, accessible via API by pertinent retailers, that connects patient food and activity choices with health care recommendations with real-time platform guidance would provide a comprehensive digital health product with features across health management, care navigation, and personal health records, that currently does not exist. Access and education are important, but behavioral change takes time and often requires bite-size interventions and incentives. Building on the functionality of Blue Button 2.0, we believe a digital health product with built-in incentives (utilizing existing Rewards and Incentives or Supplemental Benefits funding) and applied lifestyle and nutrition care standards would be more effective, enabling behavioral change through coordinated clinical and consumer data integration.

PC-7. If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

Response: CMS should implement a comprehensive data collection framework that captures both clinical and economic outcomes through integrated measurement approaches. There are various ways to measure the impact of digital health products, reductions in weight/BMI, lowering of A1c and cholesterol (output metrics), and improvements in daily choices such as nutrition and activity (as measured by assessing grocery baskets via Basket Scores or active weekly minutes, both examples of input metrics) are expected to show statistically significant improvement. Over time, reductions in spend using matched-pairs analyses (e.g., PMPM comparison) and longitudinal cost comparisons (e.g., total cost of care) can be used to demonstrate the financial impact.

Data Access and Integration

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers, or both?

a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?

b. What are specific sources, other than claims and clinical data, that would be of highest value, and why?

c. What specific opportunities and challenges exist to improve accessibility, interoperability, and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

Response: Related to PC-1 and PC-4, The most significant data gap exists between clinical health records and consumer behavior data. Patients receive nutritional guidance from healthcare providers but lack tools to connect these recommendations to their actual purchasing decisions during grocery shopping, which occurs approximately 52 times annually for the average consumer.

A digital health product like Sincerely Health captures real-time consumer purchasing data that is currently missing from healthcare analysis. Integrating this consumer data with existing healthcare records would generate meaningful clinical and risk prediction insights previously unavailable to CMS. This integration would enable CMS to evaluate which incentive structures most effectively encourage patients to follow clinical recommendations and provide comprehensive data for comparing health outcomes with actual consumer behavior.

Such a digital health product would replace unreliable self-reported dietary information with objective purchasing data, allowing CMS to accurately track the effectiveness of nutritional interventions, measure the impact of various incentive programs, and monitor actual nutritional spending patterns over time. This data would support evidence-based program refinements and better correlation between consumer behavior changes and both health outcomes and healthcare cost variations. While patients can obtain their medical records and medication summaries, getting structured guidance on lifestyle modifications required to manage their disease or mitigate future risks is lacking.

For example, the American Diabetes Association recommends at least 150 minutes per week of moderate-intensity aerobic activity, such as brisk walking, along with whole food focus and limited added sugar and starch, for adults with diabetes. Application of these recommendations varies greatly across providers. We recommend a structured framework that enables care teams to provide structured lifestyle recommendations and health management, and navigational tools. This would help patients apply these recommendations daily while transmitting compliance data or areas for additional education back to their care teams.

PC-9. Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view

as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

a. What difficulties are there in accessing or utilizing these data sources today?

b. What suggestions do you have to improve the Blue Button 2.0 API experience?

c. Is there non-CMS data that should be included in the API?

Response: The most valuable addition would be clinical outcome data from Medicare plans and provider quality reporting systems. This includes lab results, vital signs, and care plan information that would enable applications to provide personalized, actionable guidance rather than just historical claims review. Consumer-based data related to healthcare outcomes and costs is a critical next step in better understanding patient behavior and providing an opportunity to influence healthier behaviors.

Current access difficulties include the fragmented nature of clinical data across different provider systems, inconsistent data formats, and privacy restrictions that limit real-time data sharing. Developers cannot access the clinical context needed to make claims data meaningful for patient decision-making.

Blue Button 2.0 improvements should include standardized clinical data integration, real-time data updates rather than batch processing, and enhanced patient consent mechanisms that allow granular data sharing permissions. The API should also support event-driven notifications for significant health events or care plan updates.

Responses to Selected Technology Vendors, Data Providers, and Networks Topics

Ecosystem

TD-1. What short term (in the next 2 years), and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

Response: CMS could provide funding and infrastructure support to reduce development barriers for comprehensive digital health products. Building digital health products that connect consumer-driven behavior data with health care data requires substantial investment that many developers cannot afford independently. Additionally, providing incentives to pharmacy stakeholders to widely adopt FHIR standards and contribute to the enrichment of patient records, becoming an integral part of the Value-Based Care ecosystem, is important. This can be achieved by prioritizing payment structures that incentivize pharmacy data systems and interoperability investments and promoting robust and modern data sharing practices. A CMS-funded initiative to establish a comprehensive consumer-based digital health product that delivers nutritional guidance,

education, and healthy incentives and links that information with patients' health care data would allow developers in this space to properly build a next generation app that promotes innovation in consumer digital health products and encourages patient, caregiver, and provider engagement with the resulting app.

Data Exchange

TD-12. Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

Response: CMS should consider operationalizing consumer-based behavior, endorsing consumer behavior data sources that directly impact health outcomes and costs. Current healthcare records lack objective data on patient behavior outside clinical settings, relying instead on self-reported information. Systematic platforms that capture real-time purchasing decisions would provide objective insights into how patients implement clinical recommendations and support evidence-based interventions.

Evaluation criteria should prioritize data sources that demonstrate clear correlations between consumer behavior and health outcomes. Key metrics include lab values, objective scoring systems that track behavioral changes over time, such as nutritional quality of food purchases, and their correlation with healthcare utilization patterns. CMS could measure how improvements in consumer behavior translate to better treatment adherence, reduced healthcare visits, and lower costs.